Suspected Stroke - Tenecteplase (TNK) Transfer

AM-06 | Adult Medical

Quick flow

01 Verify Transfer [Paramedic]

Accepted sending/receiving plan • exact TNK bolus time known • dose/weight verified • last known well documented

02 Stabilize Before Departure [Paramedic]

Airway • glucose • baseline neuro exam • BP <180/105 • IV sites checked • written orders verified

03 Monitor Every 15 Minutes [Paramedic]

BP • GCS • pupils • focal findings • airway • SpO_2 • bleeding/angioedema symptoms

04 Complication? [Paramedic]

Severe headache • vomiting • acute hypertension • worsening deficit • seizure • bleeding • angioedema

05 Support + Notify [Paramedic / Medical Control]

ABCs • stop nonessential infusions • contact receiving stroke facility and Medical Control immediately

06 Direct Transport [EMT / AEMT / Paramedic]

NPO • cardiac monitor • no unnecessary invasive procedures • minimize delay

Overview

- Interfacility transfer after hospital-administered Tenecteplase (TNK) for suspected acute ischemic stroke.
- TNK is a single IV bolus. There is no thrombolytic infusion to continue, titrate, or stop during transport.

Provider-Level Actions

A Paramedic-led interfacility transport with focused post-thrombolysis monitoring.

EMT

- Assist airway support, monitoring, movement, NPO precautions, bleeding surveillance, and documentation under the transport-team plan.
- Immediately report neurologic decline, vomiting, new bleeding, or airway symptoms.

AEMT

- Assist with authorized monitoring and essential vascular access. Avoid unnecessary invasive procedures, IM medication, and additional vascular attempts after thrombolysis.
- Apply prolonged direct pressure after any necessary vascular access.

Paramedic

- Verify the exact TNK bolus administration time before departure; do not depart without it.
- Verify patient weight, total TNK dose, last known well, baseline neurologic examination or NIHSS when available, sending/receiving acceptance, destination, and written medication/infusion orders.
- Perform and document BP and focused neurologic reassessment at least every 15 minutes; maintain continuous cardiac monitoring and pulse oximetry.
- Continue an antihypertensive infusion only under written sending-facility orders and accepted pump parameters. Do not initiate or titrate a new antihypertensive under AM-06 without Medical Control or receiving-facility direction.

- For deterioration, support ABCs, recheck BP and glucose, contact the receiving stroke facility and Medical Control immediately, and continue direct transport unless immediate lifesaving diversion is required.

Indications

- Interfacility transfer after hospital-administered Tenecteplase (TNK) for suspected acute ischemic stroke.

Contraindications

- Do not initiate transfer until the patient has an accepted sending/receiving plan and the exact TNK bolus-administration time is known and documented.
- Do not administer antiplatelet agents, anticoagulants, additional thrombolytic therapy, or unapproved IM medication during transfer unless specifically ordered by the sending stroke physician and confirmed with Medical Control.

Assessment | Before departure

- Document exact last known well and symptom-discovery times.
- Verify patient weight, total TNK dose, and exact TNK bolus administration time; an estimate is not acceptable.
- Obtain baseline BP, GCS, pupils, focal neurologic findings, glucose, airway assessment, IV-site assessment, and NIHSS or facility neurologic examination when available.
- Confirm written medication/infusion orders, receiving physician/facility acceptance, destination, and transfer plan.

Assessment | En route

- Repeat BP, GCS, pupils, focused neurologic examination, airway assessment, and symptoms at least every 15 minutes; increase frequency for instability or active antihypertensive infusion.
- Watch for severe headache, nausea/vomiting, acute hypertension, declining consciousness, new or worsening deficit, seizure, angioedema, and external, GI, GU, or IV-site bleeding.

Treatment steps

- Maintain oxygenation and ventilation; keep the patient NPO.
- Maintain BP below 180/105 mmHg for the first 24 hours after thrombolysis. Do not use the pre-thrombolysis 185/110 threshold as the post-TNK target, and avoid intensive SBP lowering below 140 mmHg.
- Use the limb without an infusion for BP readings when practical.
- Continue only an antihypertensive infusion specifically ordered by the sending facility and accepted under the interfacility-transfer policy; titrate only to written parameters.
- Avoid unnecessary invasive procedures and IM medications. Apply prolonged direct pressure after any essential vascular access.
- For neurologic deterioration, bleeding, angioedema, seizure, severe headache, or vomiting, support ABCs, stop nonessential infusions, recheck BP and glucose, and immediately contact the receiving stroke facility and Medical Control.
- Transport directly without unnecessary delay. Consider Covenant Health Air only when it provides a meaningful time or capability advantage and does not delay definitive movement.

Medications

Tenecteplase (TNK)

Single IV bolus administered by the sending hospital

- There is no ongoing TNK infusion during transport.
- The exact bolus administration time must be known and documented before departure.

Antihypertensive infusion

Continue only under written sending-facility orders and receiving-facility acceptance

- Maintain BP below 180/105 mmHg after thrombolysis.
- Follow approved interfacility-transfer policy and pump parameters; do not begin or alter treatment without Medical Control or receiving-facility direction.

Warnings / precautions

- Sudden neurologic deterioration, severe headache, vomiting, seizure, acute hypertension, or bleeding after TNK may indicate intracranial hemorrhage or edema.
- Angioedema can threaten the airway even without other allergic findings.
- TNK is a bolus medication; there is no thrombolytic pump or remaining drug volume to manage.
- Do not delay transport for nonessential procedures or attempt to normalize blood pressure beyond the ordered post-thrombolysis target.

Clinical pearls

- Precise times - not estimates such as ‘about an hour ago’ - are essential for stroke care and must be part of the handoff.
- A stable initial examination does not eliminate the risk of hemorrhage or angioedema.
- NPO status and avoidance of unnecessary invasive procedures reduce avoidable post-thrombolysis risk.

Special populations | Wake-up stroke

- Last known well is the last time the patient was known to be at neurologic baseline, not the time symptoms were discovered.

Special populations | Airway deterioration

- Prioritize airway assessment and early airway preparation for orolingual angioedema, stridor, inability to protect the airway, or inadequate ventilation. Use the airway and anaphylaxis protocols while immediately notifying the receiving facility and Medical Control.

Related pathways / contacts

UP-04 Altered Mental Status - Open for ongoing neurologic and reversible-cause assessment.

AM-05 Hypotension / Shock - Open for hypotension or inadequate perfusion.

AM-01 Allergic Reaction / Anaphylaxis - Open for angioedema or another allergic airway emergency.

AR-01 Adult Airway Management - Open for airway protection or ventilatory support.

UP-13 Seizure - Open for new seizure or recurrent convulsive activity.

AM-04 Hypertension - Open for the defined undifferentiated-CVA BP branch only; post-TNK treatment remains governed by written facility orders.

AM-07 Crashing Patient - Open for peri-arrest deterioration.

Source and references

American Heart Association/American Stroke Association. 2026 Guideline for the Early Management of Patients With Acute Ischemic Stroke. | Tennessee EMS ALS/BLS Blended Protocol Guidelines. | Sending and receiving stroke-center transfer orders. | North Carolina College of Emergency Physicians AM-06 source protocol adapted for TNK.

Revision: October 2026 | Last verified: October 16, 2026